

sent envoys to China to study that country's culture and medicine. Direct Chinese influence on Japanese medicine, which was practiced for the most part by the monks, continued for 1,000 years. In the 16th century, Japan started to assert its cultural identity, and *kampoh* developed its own characteristic traits, emphasizing the Japanese ideals of simplicity and naturalness. However, certain Chinese concepts, such as *yin* and *yang* and *ki* (*qi*), continued to have a central role.

In 1868, the Japanese embraced Western conventional medicine. Formal training in *kampoh* officially ceased in 1885, but a few committed practitioners passed their knowledge on to younger generations, keeping the tradition alive. In the last 40 years the number of medical practitioners who use *kampoh* within their practice has greatly increased. Many Japanese medical schools now include training in *kampoh* as part of the curriculum.

Korean herbal medicine is very similar to mainstream Chinese herbal medicine, and almost all the Chinese herbs are used in Korea. Ginseng (*Panax ginseng*, p. 118) has been cultivated in Korea for home use and export since 1300.

Importance of Chinese Herbal Medicine

Since 1949 when the Communists gained control, the herbal tradition has flourished in China (see p. 29) and today it is recognized as a valid medical system, available to the Chinese on an equal footing with conventional Western medicine. As is often the case elsewhere, herbs seem to be used mainly for chronic conditions, while Western medicine is more frequently employed for serious acute illness.

Chinese herbal medicine, however, is not just of significance in China and the surrounding regions, but is practiced by trained practitioners in every continent and, in some countries, now has official government recognition. For example, there has been a traditional Chinese medicine hospital in Paris since 1996, and in 2013 the French government agreed to open three new traditional Chinese medicine hospital centers. Herbal medicine is now taught in 34 Chinese universities. This development (and the massive input of resources involved) has helped revitalize herbal medicine around the world over the past 30 to 40 years.

The impact of these developments has not been confined solely to those taking herbal medicines—whether in China or elsewhere. In 2015 Dr. Tu Youyou, a researcher trained in traditional Chinese medicine and conventional medicine, was awarded the Nobel Prize for medicine for her lifelong study of sweet Annie (*Artemisia annua*, p. 67) and its key active constituent artemisinin. From her base at the Academy of Traditional Chinese Medicine in Beijing, she proved that the herb (and its active constituent) had unusually potent antimalarial activity. Artemisinin has now become the standard treatment for acute malaria.



The *tai chi* symbol (in the center of the silk material) illustrates the harmony of *yin* and *yang*. An imbalance of these principles leads to illness.



Ju hua
(*Chrysanthemum x morifolium*, p. 78) is popular as a relaxing infusion. It also improves vision.

Corydalis rhizome

Corydalis
(*Corydalis yanhusuo*, p. 86) has a potent painkilling action.

Corydalis tincture

Zhe bei mu (*Fritillaria thunbergii*) is taken in eastern China for coughs and bronchitis.

Lycium aerial parts

Coptis (*Coptis chinensis*, p. 195) has been shown to improve tuberculosis in a clinical trial.

Fo ti
(*Polygonum multiflorum*, p. 124) is thought to concentrate *qi* (vital spirit) in its root, and is taken to improve longevity.

Shan yao
(*Dioscorea opposita*) is used in the "Pill of Eight Ingredients," a traditional Chinese remedy for diabetes.

Suan zhoa ren
(*Ziziphus spinosa*, see *Z. jujuba*, p. 283) is used in Chinese medicine to "nourish the heart and cleanse the spirit."